

NEUROLOGY PROGRESS NOTE

Follow-up visit · Established patient

Patient Alice Chen · 37F · DOB 1988-09-14
Patient ID PT-4471290
Primary diagnosis G43.709 · Chronic migraine without aura, intractable

SUBJECTIVE

The patient returns for follow-up of chronic migraine. She has maintained a headache diary for four months, averaging 17 headache days per month, of which 9 to 11 are migrainous with photophobia, phonophobia, and nausea. Attacks last 6 to 20 hours. She uses sumatriptan 100 mg for acute treatment on approximately 8 days per month with partial relief. She was started on topiramate 25 mg daily on 2026-05-12, titrated to 50 mg daily on 2026-05-26, and stopped it on 2026-06-16 after five weeks. She stopped because her out-of-pocket cost rose after a formulary change, not because of side effects; she reports she tolerated it without paraesthesia or cognitive complaints. She has not tried a beta blocker, a tricyclic, an antihypertensive, or onabotulinumtoxinA. She is not pregnant and is using contraception. There is no history of an adverse drug reaction requiring hospitalisation.

OBJECTIVE

Alert and oriented. Cranial nerves II through XII intact. Motor 5 out of 5 throughout. Sensation intact. Reflexes 2 plus and symmetric. Gait normal. Fundoscopic examination normal with sharp disc margins. No cervical or occipital tenderness. Blood pressure 118 over 74.

ASSESSMENT

Chronic migraine without aura, intractable, G43.709, at 17 headache days per month by diary over four months. One preventive agent trialed: topiramate, 50 mg daily, five weeks, discontinued for out-of-pocket cost rather than intolerance or lack of efficacy. No beta blocker, tricyclic, or other preventive class has been attempted. No contraindication to remaining preventive classes is present. No pregnancy, no prior hospitalising adverse drug reaction. Not currently on any CGRP antagonist. Acute medication use is 8 days per month, below the medication overuse threshold.

PLAN

Requested Neuvexa (obeltamab) 140 mg subcutaneously every four weeks and enrolled the patient through ArvelisConnect. I recognise the plan may require a second preventive trial. If prior authorization is not approved, initiate propranolol LA 80 mg daily and titrate to 160 mg over four weeks, continue the headache diary, and reassess at eight weeks for resubmission. Continue sumatriptan for acute attacks and monitor acute medication days. Return in eight weeks.

ELECTRONICALLY SIGNED BY	DATE / TIME
Elena Vasquez, MD	2026-08-26 14:02

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